

Health Sector Reform (HSR) Fact sheet for national managers

Fact Sheet 2 of 3

Responsibilities to be retained at national level during HSR

Regardless of the type of HSR in a country a number of functions need to be retained at national level. These normally include the following:

- *Policy setting, standards and guidelines for EPI:* These include norms and standards for infrastructure, new vaccines, safe immunization and waste management.
- *Regulation:* Legislation and regulation of the different stakeholders.
- *Strategic planning of EPI:* Development of service delivery strategies and/or a multi-year plan of action.
- *Integration and coordination:* Coordination with other programmes.
- *Joint reviews and coordination with actors at different levels:* Coordination of private providers, donors through Interagency Coordination Committees.
- *Budgeting:* Coordination of national and sub-national EPI budgeting.
- *Financing:* Ensuring a system of equitable financing and allocations for EPI.
- *Procurement:* Preparing tender documents and ensuring timely procurement.
- *Monitoring:* Disease surveillance notification and aggregation of data from lower levels (forecasts and coverage); maintaining performance monitoring with feedback to sub-national level; ensuring surveillance notification of diseases (esp. AFP), forwarding data to WHO Regional Office, and providing supervision.
- *Evaluation:* Ensuring regular immunization assessment exercises and initiating research into specific problems such as cost of raising coverage.
- *Training strategies and guidelines:* Defining skill requirements and participating in work on national curriculum.
- *Resource generation:* Ensuring sufficient and equitable distribution of personnel for immunization.

Three fact sheets on immunization and Health Sector Reform:

Fact Sheet 1 takes a broad look at HSR, briefly defines the term, examines why HSR takes place and what it aims to achieve. Different elements in the reform process are discussed, and an outline of how they may affect routine immunization is given. The aims of improving access, equity, quality, effectiveness and sustainability are briefly commented upon. Finally, a simple checklist is presented on immunization activities that have to be maintained during reforms.

Fact Sheet 2 is a guide for immunization managers at national level with emphasis on how to capitalize on the reform process, and what to watch for to avoid undesirable effects. This fact sheet is more specific and focuses on how to ensure that immunization is protected during different events in the health system calendar such as planning, implementation and monitoring and evaluation (M&E).

Fact Sheet 3 is a guide for managers at sub-national level during planning, implementation and M&E.

Planning

The major tasks are *planning for routine immunization* (annual and multi-year plan), *national immunization days (NIDs)* and *introduction of new vaccines* and other innovations. In the reform process, the following should be considered regarding national planning.

Public/private mix

Reforms often aim to build a stronger private/public partnership to ensure the best use of available resources. It is important that private and NGO providers are included and consulted in the planning process. Mapping of different stakeholders' roles and responsibilities is a useful exercise before planning takes place. The public/private partnership developed during NIDs should be exploited in routine immunization and when introducing new vaccines.

Integration of services, improving health impact and strengthening health systems

Immunization is increasingly being considered in the context of the whole health system, or as part of “*an essential package of care*”. Planning will therefore be coordinated with the planners of other disease programmes. Resources going to NIDs may also be used for strengthening routine immunization, for example cold chain, disease surveillance and human resources. Adding on activities of other programmes such as Vitamin A supplementation should also be considered. Documents on evaluations and immunization service assessments which review possible synergies between immunization and the health system should be consulted during planning. The Common Assessment Tool (CAT) for immunization links immunization activities to the rest of the health system and provides ideas on how to improve the whole health system during planning.¹

What are the priorities in immunization services?

With GAVI funds available for the introduction of new vaccines in the 75 poorest countries of the world, this issue will need to be addressed by managers, especially in low-income countries. An important question is whether new expensive vaccines should be introduced if immunization services, especially the cold chain, are not functioning satisfactorily. Is it possible to strengthen weak immunization systems while introducing new vaccines? Managers involved in immunization on a daily basis should advise politicians on this. Other evidence for policy decisions such as burden of disease (BoD) studies, results from clinical trials and cost-effectiveness studies should be taken into consideration when setting priorities.

Sustainability of services

It is important to be aware that reforms may strengthen *or* weaken sustainability in immunization services. Introduction of new vaccines, especially in a multivalent antigen package, for example Pentavalent vaccines (DTP-HepB-Hib), may more than double the vaccine cost in national immunization programmes as has been observed in Ghana (2002) and Jamaica (2003). Even if GAVI support is provided for five years, countries will need to assess if they can afford multivalent vaccine in the long term.

Planning for decentralization

Immunization services are especially vulnerable during *the period of decentralization*, which may take several years. Roles and tasks must be well understood by actors at all levels. Planning and follow-up of transfer to authority is the responsibility of national managers.

Reforms and management

During HSR the major role of the immunization managers is to capitalize on the different elements in the reform process to improve the quality of services, coverage, equity and sustainability.

¹ <http://www.who.int/vaccines-diseases/service/> Use version 2003

Equity in coverage

Managers at the central level should have an overview of district vaccine coverage data and disease outbreaks (e.g. measles) during the past year. They need to make use of such data when supporting districts to do micro-planning, giving advice on training, allocating human resources, funds and other resources for improving equity in immunization.

Planning for regulations and standards

If proper regulation of vaccines does not exist, a system should be devised to build on existing drug regulatory authorities². Six essential control functions will need to be fulfilled:

1. A published set of requirements for licensing
2. Surveillance of vaccine field performance (safety and efficacy)
3. A system of lot release
4. Use of a laboratory when needed
5. Regular inspection of manufacturers for compliance with Good Manufacturing Practice (GMP)
6. Evaluation of clinical performance through authorized clinical trials.

Sector wide approaches (SWAPs) and new Health Sector Strategic Plans (HSSP)

Budgeting for immunization may be changed with introduction of SWAPs and a new HSSP. If a policy of “an essential package of care” is introduced, the budget for immunization services may have to be re-negotiated. In such events, managers need to provide public health evidence on why it is important to combat vaccine preventable diseases. Providing figures and cost-effectiveness estimates for health gains through strengthening immunization services and introducing new vaccines will be important information for such negotiations. The Interagency Coordination Committee (ICC) may be the best forum for discussing such issues with partners (see implementation for ICC on page 4).

Implementation

Important functions of the national immunization managers during implementation are: carrying out planned activities and identifying and solving problems.

Financial allocation and procurement in a reformed system

It may be difficult to compare financial allocations before and after health sector reforms, especially if funds are channelled differently, for example through block grants to districts. It is vital that **total** financial allocations for immunization are analysed to identify changes. Securing sufficient funds and supplies to be allocated to the private sector and NGOs to fulfil new roles in immunization, may also be of special concern during reform.

Similarly, procurement of vaccines and other immunization supplies may be streamlined with procurement of drugs. There may be efficiency gains in this, but the actual procedures need to be followed closely, particularly in the early phases of reform when quality assurance should be retained. Another solution may be outsourcing of *procurement, custom clearance, storage, stock management, distribution and ordering of vaccines*. In both cases the national manager should be responsible for quality control.

² For details consult: Regulation of vaccines: building on existing drug regulatory authorities. Department of Vaccines and Biologicals. WHO/V&B/99.10.

Flow of funds under decentralization

The flow of funds from national to executing level (district and/or health facility) often proves to be difficult, particularly in the case of decentralization. *Bottlenecks* in this system should be identified and resolved as early as possible so that disbursements can be made *in a timely manner*. It is often necessary to oversee the financial flow in the health care system in general as this requires working closely with the planning department in the Ministry of Health (MOH) and often the Ministry of Local Government (MOLG) and Ministry of Finance (MOF).

Supervision of public and private providers in decentralized systems

During and after health sector reforms there is a strong need to determine and communicate responsibilities for supervision at all levels. This is a national task, and includes responsibility to provide supervision guidelines, standards, norms, and training for supervision. A regulatory framework for the private (for-profit and not-for-profit) sectors in health greatly facilitates supervision. Integrated supervision (from all members of the district health team) requires a simplified “checklist” to be used by non-immunization personnel.

Improved coordination through Interagency Coordination Committee (ICC) and SWAPs meetings

An ICC consists of the various agencies active in the health sector, and may be an important forum and a resource to immunization. It is advised that the Chairman should be a senior health official to strengthen the authority and decision-making process of the ICC. Advocacy in this group may result in a stronger focus on immunization as a whole. This is also the case with the various SWAPs processes and meetings. National immunization managers should see SWAPs meetings as opportunities to draw attention of various stakeholders to immunization.

Social mobilization after decentralization

In the context of reforms, awareness of immunization must be retained in the population and new and innovative means of social mobilization should be used. At national level the responsibility is to raise awareness through schools, radio and television, campaigns, newspapers, etc., and to provide guidance and opportunities for the district and health facility level to raise awareness, for example through the establishment of committees to improve community participation. **Note:** the introduction of fees will discourage people from seeking immunization³.

Monitoring and evaluation (M&E)

Monitoring includes following indicators through a general or vaccine-specific Management Information System (MIS), problem identification on a daily basis using informal and formal sources for information. Assessments and evaluations include periodic events such as national immunization assessments, coverage surveys, specific studies to understand changes in coverage and data quality audits.

Monitoring a decentralized and integrated system

Monitoring a vertical programme is easier than monitoring a decentralized and integrated programme. An important principle is to limit the review to essential data. This may lead to a reduction in a number of performance indicators reaching the national manager. Research and assessment of national performance will be more difficult since a number of soft data, for example related to equity, may remain at district level. Integration may also result in fewer programme-specific staff at the district and health facility level who know what to look for during monitoring. These factors should be taken into consideration when devising monitoring systems and conducting assessments and evaluations. To overcome some of these obstacles for national managers, *communication* will need to be strengthened between national, regional, district and health facility level, especially during periods of change.

Designing assessments and evaluations to capture HSR concerns

Immunization assessment tools may be used between periods of full assessments to capture important health system changes. The findings can in turn be utilized for multi-year and annual planning. Studies and evaluations and training for EPI should also be revised if they do not adequately address changes in the health system. The reform elements (Fact sheet 1) may be used as a checklist for such revisions.

Including different partners

HSR aims to bring partners closer together to improve performance. This has implications for M&E exercises where private providers (for-profit and not-for-profit) need to be brought into the MIS and supervised. Managers need to find their place in joint sector reviews and on SWAPs panels to strategize focus on M&E and supervision. Research institutions may be approached to provide evidence for policy decisions on the introduction of new vaccines to be introduced. Research on identification of strengths and weaknesses in the health systems affecting immunization quality and coverage may also be an important step in improving services.

Innovations — identifying the positive changes during reforms

M&E tends to focus on identification and solving of problems. Innovations in service quality, communications, advocacy and leadership during HSR should, however, be identified. Good work should be documented through written and/or visual material and used to advocate where appropriate.

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